



February 19, 2026

Robert F. Kennedy, Jr.,
Secretary, Department of Health and Human Services
Department of Health and Human Services
Washington, DC

RE: HHS Health Sector AI RFI

Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care [RIN 0955-AA13]

Dear Secretary Kennedy,

On behalf of Clearstep Inc. (Clearstep), thank you for the opportunity to provide comments in response to the Department of Health and Human Services' (HHS) Request for Information on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. Clearstep supports HHS's "OneHHS" efforts to align regulatory, reimbursement, and research and development strategies across the Department to promote safe, effective, and equitable AI use in clinical care.

In summary, Clearstep believes that an HHS-wide AI framework that explicitly integrates both expert-encoded systems and statistical methods is essential to maximizing patient safety, ensuring explainability and transparency, and fostering responsible innovation. Our recommendations emphasize: (1) a proportionate, risk-based regulatory posture that differentiates opaque "black-box" models from expert-encoded and hybrid systems; (2) modernized reimbursement that rewards validated AI tools for delivering measurable value; and (3) targeted research, implementation science, and public-private partnerships that bridge the gap between promising AI capabilities and safe, scalable use in clinical care.

About Clearstep Inc.

Clearstep is a leader in AI-driven healthcare access and navigation solutions, offering one of the most prevalently implemented AI self-triage systems on U.S. health system websites, as documented in a recent study in Nature's npj Digital Medicine. Clearstep also serves as the routing engine for the Defense Health Agency's Digital Front Door+, supporting a diverse beneficiary population in a large, integrated delivery environment.

Our platform is built on gold-standard clinical guidelines co-authored by Dr. Barton Schmitt, a co-creator of the Schmitt-Thompson telephone triage protocols used in the majority of U.S. nurse call centers. We integrate expert systems and statistical models including machine learning and large language models to enhance patient experience, generate actionable insights, and improve care delivery. Our predictions are designed to be explainable, interpretable, and tunable, with

every triage or navigation recommendation traceable back to validated clinical best practices and subject to independent clinician review.

Clearstep's deployments include:

- AI Triage & Navigation tools that guide individuals to appropriate levels and modes of care.
- AI Voice Automation that augments nurse advice lines and call centers by handling common call reasons and improving first-contact resolution.
- AI Optimized Scheduling that matches patient needs to available resources, integrated with major EHRs via standards-based APIs where feasible.

I. Regulation

Clearstep supports HHS's objective to establish a regulatory posture on AI that is well understood, predictable, and proportionate to risk, enabling rapid innovation while protecting patients and maintaining public trust. In our view, there is a particular opportunity for HHS to clarify expectations for non-medical-device AI tools that influence access, routing, and workflow, including digital triage, navigation, voice automation, and scheduling decision support.

A. Proportionate, risk-based oversight and transparency

We encourage HHS to explicitly distinguish between:

- Opaque "black-box" statistical models and foundation models, and
- Expert-encoded and hybrid systems, whose decision pathways are traceable to established clinical guidelines and curated content.

Regulatory expectations including documentation, transparency, and monitoring obligations should be calibrated accordingly. For AI tools used in clinical navigation and decision support, HHS can build on ONC's HTI-1 rule and Decision Support Intervention (DSI) transparency elements in 45 CFR Part 170 to:

- Require that systems be explainable, interpretable, and tunable, especially where they are patient-facing or influence access, prioritization, and/or care delivery.
- Require developers to document intended use, target population, clinical context, and known limitations, including situations where human override is expected.
- Ensure that key outputs (e.g., triage dispositions, routing recommendations) are traceable back to clinical best practice guidelines and content sources, enabling independent review by licensed clinicians and local governance bodies.

These expectations should apply both to certified health IT and to non-device AI systems integrated via APIs, with proportional tailoring based on the potential clinical impact of the tool.

B. Validation, certification, and post-market surveillance

Clearstep supports formalizing expectations for pre-deployment validation and post-deployment monitoring of AI tools that are used in clinical workflows. We recommend that HHS:

- Define baseline validation expectations for non-device clinical AI (e.g., triage, navigation, scheduling decision support), including stress-testing on edge cases, representative subpopulations, and incomplete or noisy data.
- Encourage documentation of performance metrics such as hallucination rates, data drift, under- and over-triage, and first-contact resolution for navigation and voice tools.
- Coordinate with ONC to extend certification concepts in 45 CFR Part 170 to AI-driven clinical navigation and workflow tools, so that health systems have a consistent HHS signal of quality and safety when procuring such solutions.

Post-market surveillance should mirror pharmacovigilance principles, including:

- Continuous monitoring for performance degradation, bias, and drift across populations and sites.
- Defined thresholds and processes for remediation or rollback when safety or performance concerns emerge.
- Structured mechanisms for reporting and addressing significant performance or safety incidents.

C. Privacy, de-identification, and data use for AI

Clearstep strongly supports the HIPAA de-identification standards under 45 CFR 164.514 for AI training and evaluation. To accelerate innovation, we encourage HHS to provide AI-specific guidance on:

- Expert Determination (45 CFR 164.514(b)(1)): Clarifying how Covered Entities and Business Associates can most efficiently apply this method to validate datasets for AI development.
- Lifecycle Governance: Establishing clear expectations for documenting de-identification methodologies and managing the long-term safety of datasets used in AI model evolution.

These clarifications would reduce the administrative burden on developers while ensuring that model growth does not compromise privacy or security.

II. Reimbursement

HHS payment policies and programs are a primary determinant of whether high-value AI tools can be deployed and sustained in clinical care. In our experience, current reimbursement structures often under-recognize AI's contributions to access, safety, and operational efficiency.

A. Rewarding AI-enabled value, not volume

Legacy fee-for-service payment regimes rarely recognize the cost savings and safety improvements created by validated AI tools, particularly when those benefits arise from avoided visits, optimized site-of-care selection, and administrative efficiency. Clearstep recommends that HHS:

- Expand value-based care and alternative payment models that explicitly recognize AI's role in reducing avoidable ED visits, improving triage and routing, and increasing completion of preventive and chronic care services.
- Allow providers to share in savings that are attributable to AI-enabled improvements in access, navigation, and care coordination, provided tools meet HHS-defined transparency and safety standards.

B. Liability and reimbursement clarity

Uncertainty around liability for AI-supported decisions is a significant barrier to adoption. Clearstep manages this through comprehensive insurance and indemnification protections and by ensuring that our systems are used as decision support rooted in clinical best practice. To support broader adoption across HHS programs, we recommend that HHS:

- Clarify expectations for allocation of responsibility between AI developers and providers when tools are used within labeled indications, with appropriate human oversight and documented governance.
- Consider establishing safe harbor approaches in which providers using AI tools that satisfy HHS-defined criteria for transparency, monitoring, and validation receive specified protections related to AI-driven decision support.
- Align reimbursement rules so that providers are not penalized for using robust, safety-forward AI tools that demonstrably improve outcomes and operational performance.

C. Incentivizing safety-focused, expert-encoded safeguards

Reimbursement policies should explicitly recognize the added safeguards provided by systems that integrate expert-encoded knowledge with statistical models. We recommend that HHS:

- Differentiate, in payment and program design, between “black-box-only” approaches and systems that provide traceable, clinician-reviewable logic anchored in validated clinical guidelines.
- Encourage payment models that reward the use of AI tools with clear safety guardrails, conservative fail-safes, and mechanisms for continuous oversight and tuning.

III. Research and Development

HHS is uniquely positioned to catalyze research, implementation science, and public-private partnerships that accelerate responsible AI adoption in clinical care.

A. Implementation science

Clearstep encourages HHS to prioritize implementation science that studies:

- How AI tools integrate into diverse clinical workflows (e.g., digital front doors, nurse advice lines, call centers, ambulatory and specialty scheduling).
- How governance, credentialing, and privileging models can incorporate AI-supported workflows without duplicative administrative burden.
- How AI affects patient experience, equity, clinician and staff workload, and overall system efficiency.

B. Public–private partnerships and CRADAs

Building on our work within the Defense Health Agency environment, we recommend that HHS:

- Use Cooperative Research and Development Agreements (CRADAs) and related arrangements to test AI triage, navigation, and scheduling tools across heterogeneous delivery systems, including those serving Medicare, Medicaid, and other priority populations.

C. Priority focus areas

HHS research and development investments should prioritize:

- AI Voice Automation for nurse advice lines and call centers improving first-contact resolution, narrowing access gaps, and alleviating workforce strain. Particularly distinguishing those solutions that use trusted, gold standard clinical algorithms as opposed to those that use black box statistical methods that may hallucinate, drift, provide inconsistent responses, and the basis for which recommendations cannot be evaluated.
- AI Optimized Scheduling and intake that match patients to appropriate appointments and care settings across the full range of EHR and scheduling maturity.
- Methods and tools to measure and mitigate hallucinations, bias, and data drift over time, and to empower health systems to perform ongoing AI monitoring without requiring deep in-house AI expertise.

Responses to Selected RFI Questions

1. Biggest barriers to private sector innovation and adoption

Clearstep observes several key barriers to AI innovation and adoption in clinical care:

- Legal and governance uncertainty regarding liability for AI-supported decisions in mixed human–AI workflows.
- Significant variance in EHR and scheduling maturity, limiting the ability to deploy real-time booking and advanced orchestration consistently across provider organizations.

- Limited liquidity and standardized availability of administrative and resource data, which complicates the seamless facilitation of scheduling and care navigation across the healthcare ecosystem.
- Administrative hurdles, including inconsistent credentialing and privileging requirements and fragmented internal AI review processes that slow replication and scaling of proven solutions.

2. Regulatory, payment, and programmatic priorities

We recommend that HHS prioritize:

- Standardizing referral qualification and documentation in ways that support, rather than replace, local credentialing and privileging; Clearstep’s tools, for example, standardize referral criteria and documentation while leaving credentialing authority with local governance.
- Revisiting 45 CFR Part 170 to incorporate clear certification criteria for AI-driven clinical navigation and decision support, aligned with ONC’s DSI transparency and algorithmic accountability requirements.
- Recognizing and incentivizing “safe handoff” patterns structured, context-rich handoffs from AI to human coordinators where real-time booking is not authorized or technically feasible, given EHR maturity variance.

3 & 4. Non-medical devices: legal, implementation, and evaluation issues

For non-medical-device AI tools used in clinical care, Clearstep recommends that HHS:

- Establish safe harbor concepts for providers that use AI tools meeting HHS-established transparency, safety, and monitoring criteria, with clear expectations for human oversight and scope-of-use.
- Support evaluation frameworks that emphasize hallucination rates, data drift, under- and over-triage, and first-contact resolution, alongside traditional accuracy measures, particularly for navigation and voice automation tools.
- Use contracts, grants, cooperative agreements, and prize competitions to fund robust stress-testing of AI tools across diverse clinical scenarios and populations and to develop open benchmarking frameworks and monitoring toolkits.

7. Key decision-makers and administrative hurdles

Within health systems, Clearstep typically engages CIOs, CMOs, digital leaders, access and call center leaders, and legal/compliance stakeholders. The primary administrative hurdles to AI adoption include:

- EHR and scheduling maturity variance, which constrains the ability to implement real-time booking and advanced automation uniformly.

- Fragmented governance and review processes, where multiple committees evaluate AI in the absence of a unified, risk-based framework.

In practice, AI tools must be designed to function across high-maturity environments (real-time booking, robust APIs) and low-maturity environments (asynchronous workflows and safe handoffs) to be effective across the broader HHS-influenced ecosystem.

8. Interoperability, data standards, and benchmarking

Enhanced interoperability through consistent use of FHIR-based standards would substantially widen opportunities for AI tools and accelerate innovation. Clearstep recommends that HHS:

- Promote consistent implementation of FHIR resources for appointments, scheduling, and referrals across major EHR platforms to support AI-driven triage, navigation, and real-time booking.
- Prioritize benchmarking tools and shared test scenarios for AI-driven scheduling and intake, including common metrics and test cases that link patient-facing AI interactions to provider-facing workflows.
- Ensure that administrative data necessary to facilitate scheduling across all health system service lines be made interoperable for solutions that health systems desire to put in place to provide benefit to the end patient.

Conclusion

Clearstep is committed to advancing the responsible use of AI in clinical care, with a focus on improving patient outcomes, reducing provider and staff burden, and enhancing system efficiency. We appreciate HHS's leadership in developing a coordinated, HHS-wide approach to AI that balances innovation with safety, privacy, civil rights, and public trust.

Please feel free to contact Bilal Naved (bilal@clearstep.health), should you have any questions or require additional information.

Sincerely,

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